Committee: World Health Organization (WHO)

Topic: The Question of Combating Unlicensed and Illegal Medical Procedures by Establishing Standardized Regulations

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Introduction

A wide range of medical procedures is performed around the world. To protect global health and ensure the safety of all people, the World Health Organization is committed to promoting effective and equitable healthcare by supporting enhanced regulatory standards, professional oversight, and coordinated action. Despite significant advances in modern medicine and healthcare systems, unlicensed and illegal medical procedures remain prevalent across the globe, raising serious concerns because of the harmful consequences that such practices can cause to patients.

Unlicensed and illegal medical procedures involve unqualified individuals, facilities that do not comply with regulatory requirements, the use of prohibited medical products, or treatments conducted without official authorization or contrary to applicable laws. Such activities include unauthorized cosmetic injections, surgical operations, dental treatments, and the administration of medicines. Patients may seek these services because of their lower costs, greater accessibility, or misleading advertisements that understate potential risks and present the procedures as safe and uncomplicated. However, the absence of proper medical supervision, adequate equipment, and the inability to respond quickly to emergencies can significantly increase the risk of serious consequences.

The problem has become an international one as medical services are promoted and arranged across national borders. Medical tourism allows patients to receive treatment in countries with different regulatory and licensing requirements, where certain procedures may not be legally permitted, while social media enables providers to advertise directly without clearly presenting their

qualifications or the legal status of their services. Some practitioners also operate from private homes or temporary clinics, making them more difficult for authorities to identify and regulate.

In 2024, the United States Centers for Disease Control and Prevention investigated 22 people across 11 states who experienced adverse effects after receiving counterfeit botulinum toxin injections or procedures administered by unlicensed or untrained individuals. In the United Kingdom, almost 3,000 complaints involving unregistered cosmetic practitioners were reported in 2022. However, the full extent of the problem remains difficult to determine because licensing standards, enforcement mechanisms, and reporting systems vary across countries. Therefore, clearer regulations, stronger monitoring systems, and improved international cooperation are needed to prevent unsafe and unauthorized medical procedures.

Definition of Key Terms

Medical Procedure

Medical procedures refer to actions carried out by healthcare professionals to address a patient's condition. The term describes a wide range of activities performed as part of healthcare, including medical examinations and treatments. Some procedures are intended to gather information about a patient's condition or identify the cause of a medical condition, while others aim to prevent illness, relieve symptoms, restore bodily functions, or improve overall physical well-being. These procedures should be carried out by properly trained and licensed professionals in authorized facilities using approved equipment and medical products.

Healthcare Professional

Healthcare professionals are individuals who have received specialized education, training, and qualifications to provide services related to the protection and improvement of human health. They examine patients, identify health conditions, provide treatment, perform medical procedures, or support disease prevention and recovery. The specific services they are legally permitted to provide depend on their professional qualifications, field of practice, and the laws and regulations within their jurisdiction. In many countries, healthcare professionals must also be registered or licensed by a relevant regulatory authority before carrying out certain medical activities.

Medical License

A medical license is an official authorization granted by a governmental or regulatory agency that permits a healthcare professional to practice a specific medical occupation. It is provided after the individual has completed required education, undergone clinical training, and successfully passed relevant examinations. This signifies that the practitioner meets established standards for competence and is qualified to provide medical care. The scope of practice allowed under the license varies depending on the professional's role and specialization.

Standardized Regulations

Standardized regulations are common or coordinated guidelines designed to ensure uniformity and reliability across systems or industries. In healthcare, these regulations help make medical practices more consistent by setting clear standards for healthcare professionals and medical facilities. They align with common requirements for professional training, licensing and certification, medical procedures, patient safety, patient rights, and regulatory oversight.

Unlicensed Medicine

Unlicensed medicine is defined as a medical product that has not been officially assessed or approved by the National or Regional Regulatory Authority. However, an unlicensed medicine is not necessarily illegal and may be legally used under specific conditions in accordance with applicable laws and regulations. These specific medicines play a crucial role in various areas of healthcare, including addressing the unavailability of licensed medicines, meeting the needs of patients with uncommon diseases or orphan indications, and providing early access to new treatments. According to the United Kingdom's Medicines and Healthcare products Regulatory Agency (MHRA), these products, also known as 'specials,' can be provided when a doctor, dentist, or other qualified prescriber specifically requests them for an individual patient. Due to their lack of full regulatory assessment, there are possible concerns regarding adverse reactions, inaccurate dosages, inconsistent quality, unclear expiration dates, and inadequate labeling. In these circumstances, the prescribing professional takes on additional responsibility for evaluating the appropriateness of the product and ensuring that the prescription is justified based on the patient's condition.

Falsified Medical Products

Falsified medical products are medicines, vaccines, medical devices, or other health products that intentionally give false information about their identity, composition, or source. They may contain incorrect ingredients, an inaccurate amount of active substances, or no active substance at all. Some are made to closely resemble genuine medicines, which can make them difficult for patients and healthcare workers to identify. These items may be distributed through unauthorized websites, informal markets, or unlicensed providers. Since their quality, safety, and effectiveness cannot be guaranteed, their use can lead to treatment failure, unexpected side effects, serious health problems, or death.

Regulatory Oversight

Regulatory oversight refers to the process of monitoring healthcare professionals, medical facilities, and medical services to make sure that they follow established laws and safety standards. This responsibility is usually carried out by government agencies, medical councils, or other authorized organizations. For example, the General Medical Council (GMC) in the United Kingdom regulates doctors, while state medical boards in the United States oversee medical licenses and professional conduct. Regulatory bodies may check licenses, inspect facilities, investigate complaints, and take action when rules are violated. Effective oversight helps protect patients from unsafe, unqualified, or illegal medical practices.

Medical Tourism

Medical tourism refers to the practice of individuals traveling abroad to access medical, dental, or surgical care. The term describes a range of healthcare services, such as elective procedures, surgeries, specialized treatments, and diagnostic services. People may seek medical care outside their home country for various reasons, including lower treatment costs, shorter waiting periods, access to advanced medical technology and specialized medical expertise, and the availability of treatments that may not be offered or legally permitted locally. To ensure patient safety and quality of care, these international medical activities should be provided by qualified and licensed medical professionals in accredited healthcare facilities, with clear communication of treatment information.

Background Information

Historical Development of Medical Regulation

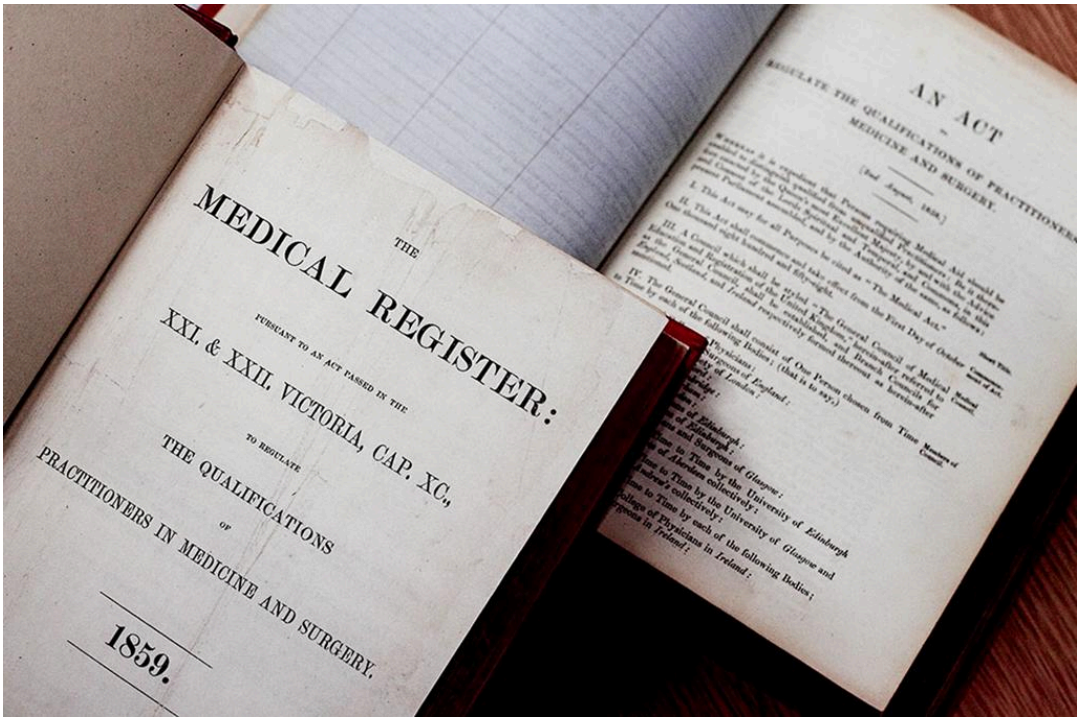
One of the earliest recorded attempts to regulate medical practice can be found in the Code of Hammurabi, established in ancient Babylon around the 18th century B.C.E. The code established specific fees for medical treatment and imposed penalties when certain surgical procedures resulted in serious injury or death. Although these penalties were severe and varied according to social status, they show that medical practitioners could be held legally responsible for the outcomes of procedures.



The Code of Hammurabi stele (Musée du Louvre, c. 1750 B.C.E.)

During the Middle Ages and before the 20th century, the regulation of medical practice remained divided with various institutions playing different roles in determining who could provide medical treatment. Modern systems of professional medical regulation began to emerge clearly during the nineteenth century, as governments became increasingly involved in establishing qualifications and registration requirements for medical practitioners. In the United Kingdom, the General Medical Council was established following the Medical Act of 1858, and the first national Medical Register was published in 1859. The establishment of such a register provided a formal mechanism for

identifying recognized medical practitioners and later developed into a broader system of professional oversight and disciplinary action.



First UK Medical Register published under the Medical Act of 1858 (General Medical Council, 1859)

Medical developments occurred in other countries. In the United States, state medical boards gradually expanded during the late nineteenth and early twentieth centuries. Medical licensing laws increasingly gave regulatory authorities responsibility for examining qualifications and for determining who could legally practice medicine. Over the following decades, these boards developed from institutions primarily responsible for issuing licenses into regulatory bodies responsible for professional discipline, competency standards, and public protection. Today, licensing also involves continued oversight of professional conduct and competence.

The development of these systems represents an important change in the regulation of medicine. Modern regulatory systems increasingly attempt to prevent harm by controlling access to medical professions, setting education and competency requirements, and restricting certain medical activities to appropriately qualified individuals. However, limitations remain, particularly in ensuring consistent enforcement, addressing regulatory differences between jurisdictions, and regulating unlicensed or illegal practices that occur beyond formal healthcare systems.

Development of Medical Licensing and Regulatory Framework

Modern health practitioner regulation generally involves several interconnected mechanisms, including professional education requirements, examinations, registration, licensing, standards of professional conduct, and disciplinary procedures. According to the World Health Organization, regulation of health practitioners is intended to reduce risks to patient safety by ensuring that practitioners meet minimum standards of competence and professional conduct. Regulatory authorities may establish educational requirements, investigate complaints, enforce disciplinary measures, and provide the public with information about registered healthcare professionals.

Internationally, there is no single licensing authority or universal medical license that applies across all countries. Instead, international organizations and regional institutions have developed frameworks intended to guide national regulatory systems and improve coordination between them. Early cooperative efforts to coordinate medical qualifications developed primarily at the regional level. In 1975, the European Economic Community adopted Council Directive 75/362/EEC, which established rules for the mutual recognition of medical diplomas, certificates, and other professional qualifications among Member States. Together with Directive 75/363/EEC, which coordinated minimum requirements for medical training, this represented an early attempt to reduce differences between national systems while allowing doctors to practice across borders. In 1993, these rules were further consolidated under Council Directive 93/16/EEC, which aimed to facilitate the movement of doctors and the mutual recognition of their professional qualifications. These earlier measures later formed part of the basis for the broader European framework established by Directive 2005/36/EC on the Recognition of Professional Qualifications.

Regional frameworks have continued to develop beyond Europe. Within the European Union, Directive 2005/36/EC currently provides a legal framework for recognizing professional qualifications obtained in other countries, including those of doctors, nurses, dentists, midwives, and pharmacists. Similar efforts emerged in Southeast Asia through the ASEAN Mutual Recognition Arrangement on Medical Practitioners, which seeks to facilitate professional mobility and cooperation among national regulatory authorities while maintaining the licensing requirements of individual countries.

More recently, international attention has expanded from the recognition of professional qualifications to the broader design of national regulatory systems. In 2024, the World Health Organization published *Health Practitioner Regulation: Design, Reform and Implementation Guidance*. The guidance identifies major regulatory functions such as establishing education

standards, setting minimum requirements for professional qualifications and behavior, applying disciplinary measures, and providing the public with information about regulated practitioners.

Major Causes and Forms of Unlicensed Medical Practice

Unlicensed medical practice does not occur for the same reasons in every country. In some regions, patients seek treatment from unqualified providers because formally licensed healthcare is difficult to access. Shortages of trained healthcare workers, geographical isolation, treatment costs, and inadequate healthcare infrastructure can leave patients with few practical alternatives. In these circumstances, informal providers may become established within local communities despite lacking recognized professional qualifications.

Bangladesh demonstrates how healthcare accessibility can contribute to this form of unlicensed practice. According to the WHO, a 2006 report found that more than 60% of the population relied on different types of unqualified health workers, particularly in communities where formally trained healthcare professionals were scarce. These providers included traditional practitioners, informal drug sellers and other community-based health workers. WHO noted that their accessibility, particularly in rural communities, was one reason patients continued to depend on them despite the potential risks associated with improvised treatment and inappropriate use of medicines. Advanced training, certification programs and improvements in access to qualified healthcare contributed to a decline in some categories of informal health workers.

In countries with greater access to formal healthcare, illegal or unregulated practice may appear in areas such as cosmetic injections, aesthetic treatments, unauthorized medicines, and procedures performed outside the legal scope of an individual's professional qualification. Providers may attract patients through lower prices, claims of rapid results, or convenient services offered outside conventional medical facilities.

As a result, unlicensed medical procedures can generally occur due to several overlapping factors. In some communities, insufficient access to licensed healthcare creates dependence on informal providers, while in other settings, growing commercial demand creates opportunities for individuals to offer medical services without sufficient training, authorization, or regulatory oversight.

These differences also affect how countries prioritize their approaches. Developed countries often place greater emphasis on stronger regulation, inspections, and enforcement against unauthorized practitioners and illegal medical products. In contrast, developing countries may consider healthcare access, workforce support, and the expansion of licensed services to be equally or

more urgent. Strict enforcement without improving access may further reduce healthcare options in underserved communities. Therefore, national priorities may differ depending on healthcare capacity, available resources, and the main causes of unlicensed medical practice in each country.

Expansion of Unregulated Cosmetic Procedures

The rapid expansion of the non-surgical cosmetic industry has created a significant concern regarding unauthorized medical practices. Procedures such as botulinum toxin injections, dermal fillers, chemical peels, and other aesthetic treatments have become accessible across various countries. Unlike major surgical procedures, many of these interventions can be offered in small clinics or private residences. This can make the distinction between regulated medical treatment and commercial beauty services less clear to consumers.

Although these treatments are often described as minimally invasive, they can still involve medical risks and require appropriate knowledge of anatomy, infection control, product handling, and the management of complications. When procedures are performed by individuals without sufficient training or outside properly regulated healthcare settings, patients may face a greater risk of adverse outcomes. In addition, smaller or temporary treatment locations may not have the same safety standards, emergency equipment, or reporting systems generally available in established medical facilities.

The growth of this sector therefore presents a regulatory challenge because the boundary between cosmetic services and medical procedures is not always clearly defined. As new techniques and products continue to develop, regulatory systems may also struggle to determine which procedures should require medical licenses, specialized training, or additional supervision.

Digital Platforms and Online Promotion of Unauthorized Medical Services

The expansion of digital platforms has further changed how medical and cosmetic procedures are promoted and arranged. Traditionally, unlicensed practitioners largely depended on local networks and direct contact with patients. Social media, online marketplaces, and messaging applications now allow providers and sellers to reach larger consumers, including individuals located in other jurisdictions.

Online promotion can also make it difficult for consumers to distinguish between qualified healthcare professionals and individuals who only appear to have professional credentials. Advertising

may focus on uploaded images, low prices, rapid results, or personal testimonials without clearly presenting professional qualifications, licensing status, potential complications, or the regulatory status of the products being offered.

Digital communication has expanded the problem beyond the physical location of an illegal clinic. An unauthorized provider, product seller, and patient may now interact through different platforms and potentially across different regions. This development creates additional challenges for conventional medical regulation, which historically developed around identifiable practitioners and physical healthcare facilities operating within clearly defined territorial boundaries.

Possible solutions

The widespread occurrence of unlicensed and illegal medical procedures has revealed inadequacies in national healthcare regulation and public safety mechanisms. To prevent unauthorized medical practices and ensure that medical processes are performed only by qualified professionals, United Nations member states are urged to cooperate in addressing the ongoing issue by strengthening regulatory frameworks and taking practical and feasible actions.

Developing WHO Guidelines and Capacity-building Programs

The WHO can support Member States by developing practical guidelines, providing training and technical assistance to prevent unlicensed and illegal medical practices. This support may include training for medical regulators and inspectors, guidance on identifying falsified medical products, and assistance in improving reporting and monitoring systems. Regional workshops and online training programs could also help countries with limited regulatory capacity strengthen their healthcare oversight.

However, countries have different healthcare systems, laws, resources, and regulatory structures. Developing countries may face difficulties in applying WHO recommendations because of limited funding, personnel, or infrastructure. Some Member States may also prefer to maintain national control over healthcare regulation. Therefore, WHO support should remain flexible and fit within existing national systems.

Establishing Enhanced Regulatory Supervision and Institutional Governance of Medical Practices

Although there are numerous countries with licensing requirements that differ for medical professionals, these standards vary significantly across national regulatory frameworks. As a result, the standards used to determine who is legally qualified to perform specific medical procedures remain inconsistent. According to WHO reports, countries such as India, Indonesia, and parts of Sub-Saharan Africa face challenges involving unclear definitions of medical practices and insufficient qualification standards, which may allow untrained individuals to provide treatment without proper

supervision. To mitigate these concerns, nations should clarify and strengthen their national licensing regulations to prevent unauthorized individuals from taking advantage of deficiencies in regulations.

Revised regulations should clearly identify which procedures require professional medical licenses and specify the necessary qualifications and practical experience for each field. Governments should also ensure transparency by clearly publishing the qualification standards, required training hours, examination requirements, and other relevant criteria used to determine whether a medical license is approved. Regular reviews of licensing can be conducted with the support of the WHO and national medical associations to ensure that regulations remain relevant to emerging treatments and technologies.

Member States could also establish regional systems for mutual recognition of medical qualifications, based on models used in the European Union. Participating countries could agree on minimum standards so that qualified healthcare professionals can have their qualifications recognized more easily across borders.

However, strengthening licensing systems may require significant financial and administrative resources. Developing countries may face difficulties in maintaining licensing databases, reviewing professional qualifications, and training regulatory personnel. In addition, differences in national laws and healthcare systems, medical education, and language requirements may make common standards or cross-border recognition difficult to apply equally in every country. Easier professional mobility may also worsen workforce shortages in countries that already have limited numbers of qualified healthcare workers. Therefore, international efforts should allow flexibility while respecting national authority over healthcare regulation.

Expanding Access to Affordable and Licensed Healthcare

Inadequate availability of accessible healthcare is one of the major factors that leads individuals to seek treatment from unqualified practitioners or unregistered medical facilities. This problem is especially serious in rural areas, low-income communities and developing countries, where shortages of qualified medical professionals and high treatment costs result in patients with few safe alternatives. Therefore, member states should cooperate to expand reliable healthcare services in underserved regions.

Financial and technical assistance from More Economically Developed Countries (MEDCs), governments, and international organizations to Less Economically Developed Countries (LEDCs) could help strengthen healthcare funding, infrastructure, and the training of medical workers in countries with limited resources. Incentives may also encourage qualified professionals to work in remote areas. By reducing financial and geographical barriers, these measures can decrease dependence on unsafe medical practices.

Enhancing Global Cooperation and Information-sharing Systems

Unlicensed practitioners may avoid legal consequences by moving across borders, using fake qualifications, or advertising unauthorized services through online platforms. Since individual countries cannot effectively address these cross-border activities on their own, nations should strengthen global cooperation and establish reliable information-sharing systems.

National health authorities, medical associations, and international organizations could exchange verified information on suspended or revoked licenses, forged qualifications, and medical facilities involved in unlawful practices. A newly formed organization may also facilitate regular meetings and provide technical guidance to support countries with limited regulatory capacity. However, shared information should be restricted to legally verified cases and handled in accordance with national laws. Through coordinated investigations and information exchange, countries can prevent unqualified practitioners from continuing unsafe activities in other jurisdictions.

However, international information sharing may raise concerns about privacy, data protection, and national sovereignty. Professional records may contain personal or disciplinary information that cannot be freely shared across borders under national laws. Incorrect or outdated information could also negatively affect a healthcare professional's reputation. Therefore, information-sharing systems should clearly define what information can be exchanged, who can access it, and how inaccurate records can be corrected.

Strengthening Monitoring and Enforcement Measures

The existence of clear medical licensing regulations does not guarantee their effectiveness if healthcare providers and medical facilities are not properly monitored. Insufficient inspections and weak enforcement may allow unqualified practitioners or unregistered facilities to continue operating

without facing appropriate consequences. All nations should strengthen the monitoring of medical services and ensure that existing regulations are consistently enforced.

National health authorities or established organizations may conduct regular and unannounced inspections of medical facilities with particular attention to data collection and transparency in order to more effectively evaluate healthcare outcomes. Governments should also establish clear reporting channels for patients and healthcare workers to report unethical practices. When violations are identified, measures such as fines, license suspension, facility closure, or criminal investigation may be applied according to national law. Cooperation among health authorities, law enforcement agencies, and professional medical associations can further improve investigations and prevent repeated violations.

Raising Public Awareness of the Risks Associated with Unauthorized Medical Practices

Many patients can receive unsafe treatment because they lack adequate information to distinguish qualified healthcare professionals from unauthorized practitioners. Misleading advertisements, false qualifications, low prices, and claims of immediate results may cause individuals to underestimate the risks of unregulated medical services. To tackle this issue, governments should strengthen public education to help patients make wise choices and develop the ability to make safe healthcare decisions.

National health authorities, medical associations, schools and online platforms should conduct awareness campaigns explaining how to verify a practitioner's qualifications, identify doubtful medical advertisements and report potentially unlawful practices. These campaigns can also explain additional risks of receiving medical interventions in private homes, hotels, or unregistered facilities. By improving public understanding of medical safety, these efforts can reduce reliance on unauthorized services and encourage patients to seek treatment from properly qualified professionals.

However, public awareness campaigns may have limited effects in communities with low health literacy, limited internet access, or poor access to licensed healthcare services. Patients may continue to use unauthorized providers if these services remain cheaper or more accessible than licensed healthcare. Therefore, public education should be combined with practical improvements in healthcare access and simple methods for checking professional qualifications.

Major parties involved

European Union

The European Union has established regional regulations to manage the qualifications of healthcare professionals working across member states. Under the Professional Qualifications Directive, doctors, nurses, dentists, midwives and pharmacists must meet common training requirements before their qualifications can be recognized in another EU country. The EU also operates an alert mechanism through the Internal Market Information System, requiring national authorities to share information when healthcare professionals are restricted or prohibited from practising or found to have used falsified qualifications. This system helps prevent unqualified or suspended practitioners from avoiding disciplinary measures by moving across borders. However, differences in national enforcement standards continue to create challenges in maintaining patient protection throughout the region.

United States of America

The United States has a decentralized medical licensing system in which state medical boards determine whether physicians are qualified to legally provide medical services and investigate violations of ethical and clinical standards. These boards also share licensing and enforcement information through the Federation of State Medical Boards (FSMB), helping authorities identify practitioners whose licenses have been suspended or revoked in other states. The public can also access physician profiles through state medical board online licensure verification databases, which include license status and disciplinary actions.

Federal agencies also take action against illegal medical products and procedures. In 2025, the U.S. Food and Drug Administration (FDA) issued 18 warning letters to websites that illegally marketed unapproved and mislabelled botulinum toxin products. In addition, federal enforcement efforts are carried out nationwide in coordination with agencies such as the Department of Justice and U.S. Customs and Border Protection, targeting the importation and distribution of counterfeit or unapproved injectable products across various states. These actions include seizures of products, criminal prosecutions, and civil penalties against individuals and businesses involved in the illegal cosmetic medical market.

United Kingdom of Great Britain and Northern Ireland

The United Kingdom maintains a national medical licensing system through the General Medical Council (GMC). The GMC investigates concerns regarding unlawful practice and patient safety. When serious violations are identified, a doctor's license may be restricted or suspended, and disciplinary actions are published on the online medical register.

The country has also taken steps to strengthen regulations for non-surgical cosmetic procedures. In response, research from University College London (UCL) found that 68% of cosmetic injectable practitioners in the United Kingdom were not medical doctors, although some held other healthcare qualifications. In collaboration with local authorities and regulators such as the Care Quality Commission (CQC), the Department of Health and Social Care (DHSC) has committed to implementing a formal licensing framework that sets stricter requirements for qualifications, training standards and infection control. These are intended to reduce the risks associated with unregulated practice and improve overall patient safety in the cosmetic treatment sector.

People's Republic of China

China has strengthened regulation of its rapidly expanding medical industry, which has raised patient safety concerns. In a nationwide enforcement campaign, Chinese authorities took action against more than 2,700 violations, revoked the licences of 28 medical cosmetology providers and ordered 47 training institutions to stop offering illegal services. The government also launched additional joint campaigns involving the National Health Commission and other agencies to investigate unlicensed cosmetic procedures, false advertising and the use of unauthorized medical products. China has also classified medical cosmetology advertisements by placing them under stricter qualifications and content requirements.

People's Republic of Bangladesh

Bangladesh has historically struggled with the widespread dependence on unregulated and unlicensed healthcare providers, especially in rural areas where access to formally trained medical professionals is limited. WHO reports that in the past, over 60% of the population relied on various informal providers, including local community practitioners. A collaborative evaluation conducted by WHO and the Ministry of Health and Family Welfare (MOHFW) estimated that there were about 33

licensed healthcare workers and 16 unqualified providers per 10,000 people. In response, both governmental bodies and NGOs have introduced training initiatives, certification systems and capacity-building programs aimed at improving the skills of informal practitioners. Consequently, the number of traditional birth attendants and healers declined significantly, dropping from 97 to 6 per 10,000 people between 2007 and 2020.

Kingdom of Thailand

Thailand has developed a large medical tourism industry, attracting a significant number of international patients seeking cosmetic procedures, elective surgeries and other medical services. However, illegal cosmetic clinics, unlicensed practitioners and the use of unregistered medical products have raised serious concerns. In 2023, Thai authorities investigated four cosmetic clinics and seized 943 items, including more than 779 cosmetics improperly used for injection, over 109 unregistered medicines, and incorrectly labeled products. Under the Medical Profession Act, medical practitioners must be registered with the Medical Council of Thailand, which has the authority to suspend or revoke licenses when regulations are violated. The government has also carried out joint inspections involving public health authorities, the Thai Food and Drug Administration, and law enforcement agencies to detect unauthorized medical procedures and illegal products within the medical tourism sector.

Republic of India

India regulates medical practice under the National Medical Commission Act, which requires doctors to be properly qualified and registered. However, in rural and low-income areas, where access to formal healthcare is limited and there is a shortage of licensed doctors, unqualified practitioners are still widely relied upon as primary healthcare providers. This situation contrasts with many other countries, where illegal medical practice is more commonly associated with areas such as cosmetic procedures.

In addition, the Clinical Establishments (Registration and Regulation) Act of 2010 establishes minimum standards for healthcare facilities. Nevertheless, its implementation is not equally applied across all states, and enforcement remains inconsistent, further contributing to gaps in access to regulated healthcare.

Federal Republic of Nigeria

Nigeria has one of the largest health workforces in Africa. However, the country faces significant challenges related to unlicensed medical practitioners and the unequal distribution of qualified healthcare professionals. While cities like Abuja and Lagos have higher concentrations of licensed doctors, many rural regions suffer from limited access to safe and regulated healthcare services.

Under the Medical and Dental Practitioners Act, medical professionals must be registered and licensed by the Medical and Dental Council of Nigeria before performing medical procedures. However, enforcement remains difficult due to workforce shortages, which can contribute to informal or unlicensed medical practice in underserved areas.

Nigeria's healthcare system is also affected by the imbalance in health workforce distribution, which undermines equitable access to care and highlights the need for improved rural healthcare systems and better deployment of medical health workers.

Republic of Korea

The Republic of Korea maintains a strict national licensing system for medical practitioners. Under Article 27 of the Medical Service Act, unlicensed individuals are prohibited from performing medical procedures, while licensed practitioners may only practice within the scope of their licenses. Nevertheless, cases involving unlicensed cosmetic procedures and proxy surgery have raised concerns regarding patient safety and accountability. The Act provides for the suspension or cancellation of medical licenses when professionals allow unqualified individuals to perform surgery or other risky procedures. Medical institutions performing surgery on sedated patients are required to install closed-circuit television (CCTV) systems in operating rooms, primarily to enhance patient safety, to ensure transparency, and to protect patients' rights.

Timeline Of Events

Date	Description of Event
WMA adopts the International Code of Medical Ethics, October 1949	The WMA established international ethical standards for physicians and professional medical conduct.
EEC adopts medical qualification directives, June 1975	European states introduced rules for recognizing medical qualifications and coordinating medical training standards.
European medical qualification rules are consolidated, April 1993	Earlier European rules were combined to make it easier for qualified doctors to work in other member states.
EU adopts Directive 2005/36/EC, September 2005	The EU established a broader system for recognizing professional qualifications across Member States.
ASEAN signs the Mutual Recognition Arrangement on Medical Practitioners, February 2009	ASEAN created a regional framework for the mobility and recognition of qualified medical practitioners.
WHO adopts the Global Code on International Recruitment of Health Personnel, May 2010	WHO Member States established principles for international cooperation on the recruitment and movement of health workers.
England establishes a legal basis for cosmetic procedure licensing, April 2022	The Health and Care Act enabled licensing of practitioners and facilities providing certain cosmetic procedures.
WHO conducts a global review of health practitioner regulation, January 2024	The review examined different regulatory systems to support the development of a global framework.
U.S. investigates harmful botulinum toxin injections, April 2024	Authorities investigated harmful reactions linked to unlicensed providers and unsafe treatment settings.

WHO publishes global health practitioner regulation guidance, September 2024	WHO introduced global guidance for strengthening health practitioner regulation and patient protection.
England confirms further regulation of cosmetic procedures, August 2025	The government announced stronger restrictions and licensing measures for non-surgical cosmetic procedures.
FDA acts against illegal botulinum toxin sales, November 2025	The FDA took action against websites illegally marketing unauthorized botulinum toxin products.

UN Involvement, Resolutions, Treaties and Events

The United Nations has played an important role in developing international standards for health practitioner regulation, patient safety, and the control of unauthorized medical products. However, medical licensing and the regulation of medical procedures remain under national authority. As a result, international efforts mainly focus on providing a common regulatory framework, improving cooperation among national authorities, and supporting stronger healthcare infrastructure. Regional organizations have also developed frameworks to recognize medical qualifications across borders and to coordinate professional standards. These arrangements aim to facilitate legitimate medical practice while maintaining national licensing requirements and protecting patients from unauthorized or unqualified practitioners.

WHO Documents and Resolutions

- WHA57.19 – International Migration of Health Personnel, May 2004

  The World Health Assembly recognized the challenges created by international health worker migration and encouraged cooperation between countries to strengthen health systems and manage the movement of qualified health personnel.

- WHA65.19 – Member State Mechanism on Substandard and Falsified Medical Products, May 2012

  The World Health Assembly established a mechanism for international cooperation to prevent, detect, and respond to substandard and falsified medical products and to strengthen national and regional regulatory capacity.

- WHO Global Surveillance and Monitoring System, 2013

  WHO introduced the Global Surveillance and Monitoring System to help national regulatory authorities report and share information on suspected substandard and falsified medical products across countries.

- WHO Health Practitioner Regulation Systems Review, January 2024

  WHO reviewed health practitioner regulation systems across countries, examining issues including professional regulation, cross-border service delivery, workforce mobility, and the growing role of unregulated health workers.

- WHO Health Practitioner Regulation: Design, Reform and Implementation Guidance, September 2024

  WHO published the first global guidance specifically focused on health practitioner regulation, addressing education standards, professional competence, disciplinary systems, and public access to information about regulated practitioners.

Cross-Border Licensing and Professional Recognition

- EEC Directive 75/362/EEC, June 1975

  The European Economic Community introduced rules for the mutual recognition of medical diplomas, certificates, and professional qualifications among Member States.

- EEC Directive 75/363/EEC, June 1975

  The European Economic Community established minimum requirements for medical education and training to support consistent professional standards.

- Council Directive 93/16/EEC, April 1993

  Earlier European rules on medical qualifications were consolidated to make it easier for qualified doctors to practice in other Member States.

- EU Directive 2005/36/EC on the Recognition of Professional Qualifications, September 2005

  The European Union established a broader framework for recognizing professional

qualifications, including those of doctors, nurses, dentists, midwives, and pharmacists, while maintaining national licensing requirements.

- ASEAN Mutual Recognition Arrangement on Medical Practitioners, August 2008

  ASEAN Member States created a regional framework to facilitate the recognition and cross-border practice of qualified medical professionals while allowing each country to maintain its own registration and licensing requirements.

Illegal and Falsified Medical Products

- WHA41.16 – Rational Use of Drugs, May 1988

  The World Health Assembly requested WHO to support programs for preventing and detecting the export, import, and smuggling of falsely labelled, counterfeit, and substandard pharmaceutical products.

- WHA47.13 – Implementation of WHO's Revised Drug Strategy, May 1994

  The World Health Assembly encouraged WHO support for Member States to combat counterfeit medicines and strengthen national regulatory systems.

- EU Falsified Medicines Directive, June 2011

  The European Union introduced measures to prevent falsified medicines from entering legal supply chains and strengthened requirements for the distribution and verification of medicinal products.

- Council of Europe MEDICRIME Convention, October 2011

  The MEDICRIME Convention established criminal law measures against counterfeit medical products and related violations while promoting victim protection and international cooperation.

Strengthening International Cooperation and Regulation

- WHO Member State Mechanism on Substandard and Falsified Medical Products, 2012

  The mechanism creates a framework for international cooperation, allowing Member States to exchange information, develop regulatory tools, strengthen supply chain oversight, and coordinate responses to unsafe or falsified medical products.

- WHO National Reporting Instrument under the Global Code, 2010

  Member States are encouraged to report information on health worker recruitment and migration, allowing countries to share comparable data and monitor implementation of the WHO Global Code.

- WHO Global Guidance on Health Practitioner Regulation, September 2024

  The guidance aims to strengthen national regulatory institutions and reduce gaps between regulatory policy and actual practice while recognizing differences in healthcare systems, workforce availability, and national circumstances.

Bibliography

Anand, Sudhir, and Victoria Fan. *The Health Workforce in India*. World Health Organization, 2016, https://iris.who.int/server/api/core/bitstreams/319faee2-09d0-4090-8427-2db98abb9547/content. Accessed 2 Aug. 2026.

Association of Southeast Asian Nations. "ASEAN Mutual Recognition Arrangement on Medical Practitioners." *ASEAN*, 26 Feb. 2009, https://asean.org/asean-mutual-recognition-arrangement-on-medical-practitioners/. Accessed 25 Oct. 2026.

"Authorities Crack Down on Improper Cosmetic Surgery Procedures." *National Health Commission of the People's Republic of China*, 10 Jan. 2019, https://en.nhc.gov.cn/2019-01/10/c_74151.htm.

"Automatic Recognition." *European Commission*, European Union, 2023, https://employment-social-affairs.ec.europa.eu/policies-and-activities/skills-and-qualifications/recognition-professional-qualifications/automatic-recognition_en.

"CDC Issues Alert for Adverse Effects Linked to Counterfeit or Mishandled Botulinum Toxin Injections." *Centers for Disease Control and Prevention*, 25 Apr. 2024, https://www.cdc.gov/locs/php/messages/2024/2024-04-25-adverse-effects-counterfeit-mishandled-botulinum-toxin.html.

"Clinical Establishments (Registration and Regulation) Act, 2010." *Clinical Establishments*, Ministry of Health and Family Welfare, Government of India, https://clinicalestablishments.mohfw.gov.in/. Accessed 26 July 2026.

"Consultation Launched into Unregulated Cosmetic Procedures." *GOV.UK*, 2 Sept. 2023, https://www.gov.uk/government/news/consultation-launched-into-unregulated-cosmetic-procedures.

"Crackdown on Unsafe Cosmetic Procedures to Protect the Public." *GOV.UK*, Department of Health and Social Care, 6 Aug. 2025, https://www.gov.uk/government/news/crackdown-on-unsafe-cosmetic-procedures-to-protect-the-public. Accessed 25 Oct. 2026.

"Definitions of Substandard and Falsified (SF) Medical Products." *World Health Organization*, https://www.who.int/teams/regulation-prequalification/incidents-and-SF/background/definitions.

"Directive 2005/36/EC of the European Parliament and of the Council of 7 September 2005 on the Recognition of Professional Qualifications (Text with EEA Relevance)." *EUR-Lex*, 30 Sept. 2005, pp. 22–142, https://eur-lex.europa.eu/legal-content/EN/TXT/?uri=CELEX:32005L0036.

"Directive 2011/62/EU of the European Parliament and of the Council of 8 June 2011." *EUR-Lex*, 1 July 2011, https://eur-lex.europa.eu/legal-content/EN/TXT/?uri=CELEX:32011L0062. Accessed 24 Oct. 2026.

"FDA Warns Companies over Illegal Marketing of Botox and Related Products." *U.S. Food and Drug Administration*, 5 Nov. 2025, https://www.fda.gov/news-events/press-announcements/fda-warns-companies-over-illegal-marketing-botox-and-related-products.

General Medical Council. "The History of the General Medical Council." *GMC*, https://www.gmc-uk.org/about/who-we-are/our-history. Accessed 20 Oct. 2026.

"Governance and Cooperation." *European Commission*, European Union, https://employment-social-affairs.ec.europa.eu/policies-and-activities/skills-and-qualifications/recognition-professional-qualifications/governance-and-cooperation_en. Accessed 20 July 2023.

"Health Workforce—Nigeria Health System and Services Profile." *African Health Observatory Platform*, 7 Aug. 2025, https://ahop.aho.afro.who.int/download/health-workforce-nigeria-health-system-and-services-profile/.

"Health and Care Act 2022: Section 180, Licensing of Cosmetic Procedures." *UK Public General Acts*, 2022, https://www.legislation.gov.uk/ukpga/2022/31/section/180. Accessed 10 Sep. 2026.

"Information for Consumers." *Federation of State Medical Boards*, https://www.fsmb.org/u.s.-medical-regulatory-trends-and-actions/guide-to-medical-regulation-in-the-united-states/information-for-consumers/. Accessed 20 July 2026.

Mahendradhata, Yodi, et al. *Indonesia*. Asia Pacific Observatory on Health Systems and Policies, https://apo.who.int/docs/librariesprovider15/publications/comparative-country-studies/indonesia-comparative-country-studies.pdf. Accessed 2 Aug. 2026.

"MDCN Act Other Regulation." *Medical and Dental Council of Nigeria*, https://www.mdcn.gov.ng/page/about-us/mdcn-act-other-regulation.

"Medical and Dental Practitioners Act." *Medical and Dental Council of Nigeria*, https://www.mdcn.gov.ng/public/storage/ckeditor/upload_7405511.pdf. Accessed 26 Oct. 2023.

"Medicine Shortages & Unlicensed Use." *Uniphar*, https://uniphar.com/globalsourcing/medicine-shortages-unlicensed-use/.

Musée du Louvre. "The Code of Hammurabi." *Louvre*, https://www.louvre.fr/en/the-code-of-hammurabi. Accessed 25 Oct. 2026.

National Medical Commission. "NMC Act." 2020, https://www.nmc.org.in/nmc-act/.

Norree, Thinakorn, et al. "Medical Tourism in Thailand: A Cross-Sectional Study." *Bulletin of the World Health Organization*, vol. 94, 2016, pp. 302–306. https://doi.org/10.2471/BLT.14.152165.

Republic of Korea. *Medical Service Act*. Act No. 19818, 31 Oct. 2023, enforced 1 May 2024. Korean Law Information Center, Ministry of Government Legislation, https://www.law.go.kr/LSW/eng/engLsPdfPrint.do?ancNo=19818&ancYd=20231031&docSelectId=pdf&lsiSeq=255803&save=save. Accessed 3 Aug. 2026.

"Supply Unlicensed Medicinal Products ('Specials')." *GOV.UK*, Medicines and Healthcare Products Regulatory Agency, 11 May 2023, https://www.gov.uk/government/publications/supply-unlicensed-medicinal-products-specials/supply-unlicensed-medicinal-products-specials.

"System for the Recognition of Professional Qualifications." *EUR-Lex*, 10 Aug. 2022, https://eur-lex.europa.eu/EN/legal-content/summary/system-for-the-recognition-of-professional-qualifications.html.

Taylor-Smith, Kelly. "Almost 70% of Cosmetic Injections Not Administered by Doctors." *Environmental Health News*, 3 Aug. 2023, https://www.cieh.org/ehn/public-health-and-protection/2023/august/almost-70-of-cosmetic-injections-not-administered-by-doctors/.

"The Code of Hammurabi." *The Avalon Project*, Yale Law School, https://avalon.law.yale.edu/ancient/hamframe.asp. Accessed 20 July 2026.

The Council of Europe. "The MEDICRIME Convention." *Council of Europe*, https://www.coe.int/en/web/medicrime/the-medicrime-convention. Accessed 23 Oct. 2026.

"ตำรวจสอบสวนกลางร่วม อย. และ สบส. บุกค้นคลินิกเสริมความงาม ใช้เครื่องสำอางฉีดแทนยา, ยาไม่มีทะเบียน และเครื่องสำอางไม่มี อย. กว่า 943 รายการ." *สำนักงานคณะกรรมการอาหารและยา*, 22 Sept. 2023, https://cemc.fda.moph.go.th/press-release/pr-60.

World Medical Association. “WMA International Code of Medical Ethics.” *World Medical Association*, Oct. 1949, https://www.wma.net/policies-post/wma-international-code-of-medical-ethics/. Accessed 22 Oct. 2026.

World Health Organization. “From Unqualified to Professional Medicine: Bangladesh, a Promising Journey.” *World Health Organization*, 6 Apr. 2021, https://www.who.int/bangladesh/news/detail/06-04-2021-from-unqualified-to-professional-medicine-bangladesh-a-promising-journey.

World Health Organization. *Global Patient Safety Action Plan*. https://www.who.int/teams/integrated-health-services/patient-safety/policy/global-patient-safety-action-plan. Accessed 18 July 2026.

World Health Organization. *Global Surveillance and Monitoring System for Substandard and Falsified Medical Products*. 30 Jan. 2017, https://www.who.int/publications/i/item/9789241513425. Accessed 25 Oct. 2026.

World Health Organization. *Health Practitioner Regulation: Design, Reform and Implementation Guidance*. World Health Organization, 2024, https://www.who.int/publications/i/item/9789240095014.

World Health Organization. “International Migration of Health Personnel: A Challenge for Health Systems in Developing Countries.” *WHA57.19*, May 2004, https://apps.who.int/gb/ebwha/pdf_files/wha57/a57_r19-en.pdf. Accessed 25 Oct. 2026.

World Health Organization. “National Reporting Instrument.” *World Health Organization*, 19 May 2021, https://www.who.int/news-room/questions-and-answers/item/national-reporting-instrument. Accessed 25 Oct. 2026.

World Health Organization. “Substandard/Spurious/Falsely-Labelled/Falsified/Counterfeit Medical Products.” *WHA65.19*, 26 May 2012, https://www.who.int/publications/i/item/A65_R19. Accessed 25 Oct. 2026.

World Health Organization. *Medicines in Health Care Delivery: Thailand—Situational Analysis, 23 November–4 December 2015*. World Health Organization Regional Office for South-East Asia, Feb. 2016, https://cdn.who.int/media/docs/default-source/searo/hsd/edm/csa-thailand-situational-assessment-2015.pdf. Accessed 2 Aug. 2026.

World Health Organization. *WHO Global Code of Practice on the International Recruitment of Health Personnel*. 20 May 2010, https://www.who.int/publications/i/item/wha6316. Accessed 25 Oct. 2026.